SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Continuous fluorouracil + radiotherapy

Locally advanced rectal cancer · NCCP 00421 · version 6 · 7-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

Schedule
Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity

Pelvic radiotherapy
LOCAL RADIATION TREATMENT

- ☐ Bowel irritation / diarrhoea
- ☐ Pelvic skin soreness
- ☐ Urinary symptoms
- ☐ Fatigue
- ☐ Possible lasting bowel / bladder effects
- ☐ Sexual function / fertility effects

Confirm treatment aim, expected benefit, reasonable alternatives and no-treatment/supportive-care options. Discuss DPD, infection, mouth/bowel/skin toxicity, cardiac symptoms, fertility, contraception, breastfeeding and interactions. Discuss separate radiotherapy consent and possible late bowel, bladder, sexual and fertility effects. Explain line/pump risks and response to malfunction.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00421 v6.

SACTCheck v0.84.32
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